

# 3

## Time Management and Administrative Simplification: Running Your Practice Efficiently and Effectively!

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By Mary Jean Sage, Sage & Associates

Medical practices are struggling to survive in an industry beset by constant change and declining operating margins. With Medicare physician reimbursement projected to decrease 30 percent over the next several years and major health plans unwilling to negotiate contract rates that cover the cost of care, every practice must be performing both efficiently and effectively to ensure financial viability and quality care for patients. Efficiency—accomplishing goals without waste or loss—lowers costs and increases your return per unit. Effectiveness—producing desired results—increases quality outcomes and patient satisfaction. When a practice is running efficiently and effectively, the physician is free to focus on seeing patients rather than dealing with administrative hassles.

According to Owen Dahl, author of *Think Business! Medical Practice Quality, Efficiency, Profits*, efficiency for a medical practice hinges on the goal of providing value to a patient in every single encounter—be it face-to-face time with the physician or a phone call to schedule an appointment, get a prescription refill, or ask about lab results.

In today's environment, it is critical to control your costs. In reviewing each service your practice provides, you must consider these questions: Does the service bring value to my practice? Are my customers pleased with this service? How much does this service cost? How can this service be provided more efficiently to reduce costs and increase quality of care?

The practice must provide this value in a cost-effective way or you will not be able to stay in business. Therefore, it's crucial to review how day-to-day work is performed and figure out how to optimize results. Generally, completing in-house tasks can involve multiple staff members and myriad steps, so you'll need to look at each process from start to finish. Ask questions. Often, certain steps have been integrated into a process and passed along as gospel with no one having any idea why. When asked, a worker may say, "We've always done it that way." Make sure staff knows and understands why each component of a task is necessary. This is also the perfect time to write down the particulars of each process and create or update a procedures manual.

In this chapter, we will focus on identifying errors, delays, bottlenecks, and inefficiencies through practice assessment. We will discuss how these errors and inefficiencies contribute to reimbursement problems and loss of productivity and give pointers and tools for standardizing processes and improving customer satisfaction and office flow.

### LOOKING AT YOUR PRACTICE

The first step in improving efficiency and effectiveness is identifying where your practice is succeeding and where there is room for improvement. There is no better way to determine that than to perform a self-assessment of the practice. During your assessment, you will want to cover the following:

- Defining roles
- Front-end processing
  - Accuracy of information collected
  - Appointment scheduling
  - Check-in
  - Failure to collect
  - Patient wait times
  - Check-out
- Late arrivals and no-shows
- Managing call volume
- Operational and patient flow
  - Utilizing the Internet
  - Clinical services/procedures
  - Billing systems
  - Other systems as appropriate
- Patient satisfaction (for more on patient satisfaction, please see Chapter IV)
  - Surveys
  - Results
  - Goals
- Avoiding common traps and pitfalls

## THE ASSESSMENT

Conducting an assessment of your practice does not have to be complicated. Focus on one process at a time (including interrelated work) and assess each aspect from start to finish. Generally this entails these basic steps:

1. Collect all existing written documentation about the process.
2. Inform staff of your goals and conduct an interview with each staff person directly involved in the process.
3. Observe processes as they are performed in real time.
4. Participate in or perform the task yourself.
5. Develop recommendations.

Below we've provided a sample road map for conducting an assessment of two essential procedures: patient registration and check-in. While it could be followed just as written by most practices you will gain far more knowledge and understanding

of your office workings by creating your own guidelines, using the five steps outlined above.

Part of this assessment will require you to pretend you are a patient undergoing registration and check-in. This is an important opportunity for you to experience what it is like to be a patient in your practice. Viewing things from a different perspective can be very useful as you work to improve the efficiency of your practice.

Before you begin, inform staff about your goal and instruct them to act in the normal fashion. Make sure they understand you are assessing processes and not anyone's individual performance. Write down each of the steps so you have a road map to follow after the assessment is complete.

### Step I—Locate Documentation

Find and review any documentation you have for check-in and registration procedures. Make a list of all forms and paperwork and keep it all in a file for easy access. Your list should include:

- Job descriptions
- Registration forms
- Payment rules
- Pre-authorization requirements
- Authorization process
- Other clinic rules

### Step II—Interview Staff

After you have reviewed the above documentation, you can begin interviewing each staff member directly involved in patient registration and check-in. Ask staff to walk you through each phase of the process. Details are important! After you have completed the interviews, give employees the opportunity to comment or suggest improvements.

Take detailed notes. Information and feedback from these interviews serves several important purposes:

- Testing staff job knowledge
- Identifying bottlenecks and other inefficiencies
- Creating new or updated job descriptions
- Creating a new or updated procedures manual
- Creating new or updated education and training tools
- Creating or updating patient survey questions
- Encouraging staff comment and recommendations

### Step III–Observation

Next, watch staff as they conduct patient registration and check-in. Observe these processes twice a week for several weeks during peak clinic hours. While this process is normally the purview of the office manager or administrator, the physician can and should be involved in how it is performed.

Listen, watch carefully, and document any bottlenecks, inconsistencies, or other issues. Remember, you need to pay attention to the time each task takes to complete. Try not to distract or disrupt what would be the normal routine. Ask questions when necessary or to clarify something you may have missed. This is not the time to make suggestions or change current processes; wait until it is time for your final assessment in Step V.

### Step IV–Participation

It is important that you are able to perform the tasks associated with registration and check-in by following the documentation you have on hand.

You should also take part in the process as if you are a new patient. Your employees should experience the process from that vantage point too. Have different staff members engage in this role-play during peak business hours several times over the course of one month. Each staffer should sit in the waiting room and complete all the forms patients are required to complete. Take note of any questions on the forms that are not necessary.

- Honestly assess your waiting room for the following:
  - Comfort
  - Cleanliness
  - Reading materials
  - Seating
  - Water
  - Restroom availability
  - Lighting
  - Medical/educational literature/pamphlets
- Observe other people in the waiting room and note any relevant remarks overheard.
- Complete the forms as any new patient would.
  - Note the time it takes from the moment you arrive at the receptionist window until you sit down to begin completing forms.
  - Complete the forms as though you are at your own physician's office.

- Document the total time it takes you to complete the forms.
- Once you've returned the forms to the receptionist, note the amount of time it takes the receptionist to enter the information from the forms into the practice management system and ready your chart for rooming.
- Now, note the amount of time it takes for to be escorted to an exam room.

### Step V–Create Recommendations and Implement Change

Once you've completed the previous steps, deficiencies will have become very apparent. Now it is time to do something with the results. Make sure each task or service you've observed is necessary, brings value to your practice, and is performed well in a cost effective manner. As a final step, to complete your assessment:

- Analyze background gathered.
- Review notes taken during the interview, observation, and participation phases.
- Outline your recommendations.
- Determine the cost of any changes. Will it be cost-effective and add value to make the recommended changes?
- Meet with staff to discuss and fine-tune recommendations.
- Formalize and implement recommendations.
- Monitor progress.
- Create goal-oriented incentives. For example, if you are having problems with your scheduling (schedules aren't full, you are frequently double booked, etc.), offering an incentive to correct these issues can be much more motivating than corrective action.

Acknowledge areas where the practice is performing well. There is no surer way to motivate your staff than to praise them when a job is well done.

On the flip side, also point up areas where there is room for improvement and processes that need to be reorganized or restructured. You will need to develop plans to bring these areas into compliance with your goals for the practice, its operation, its quality, and its profitability. Your goal should be to look at the entire practice within a six-month period.

Some practices, rather than conduct this assessment all by themselves, will ask a medical practice management consultant to perform it or offer guidance. Your state medical association, local medical association, or professional society may be able to refer you to practice consultants in your area (see Chapter II). The Medical Group Management Association (MGMA, [www.mgma.org](http://www.mgma.org)) and National Society

of Certified Healthcare Business Consultants (NSCHBC, [www.smdmc.org](http://www.smdmc.org)) can also direct you to such advice.

As you formulate recommendations for change, remember that while you can cut costs by reducing employee time, it's not time to start handing out pink slips. Being overstaffed is obviously a financial drain, but before making changes you need to make sure all employee departments are functioning as they should. While you may be overstaffed in one area, you may be understaffed in others; for example, you may find that one of your front office workers has time on her hands, but your billing and collection staff is clearly shorthanded, judging from the volume of accounts over 90 days. In this instance you may want to consider shifting some of your resources from the front office to the back.

## IDENTIFYING AND ELIMINATING VARIANCES

Variance in the way tasks are performed can contribute significantly to breakdowns in processes, which can create practice inefficiencies. The rest of this chapter focuses on recommendations that will help eliminate these variances, increase practice productivity, and reduce rework.

### Defining Roles

Many problems in physician practices can be traced back to a lack of written job descriptions, policies, and procedures. Compounding these problems are high staff turnover, inadequate training, and lack of communication. Failure to maintain employee job descriptions and policies and procedures manuals severely limits a business's ability to achieve optimal efficiency.

Successful businesses maintain manuals that clearly define employee roles and provide step-by-step instructions on how to complete tasks. Such manuals and instructions will allow you to quickly identify system breakdowns, measure quality of work and productivity, evaluate staffing levels, hold employees accountable, and prevent unnecessary work and other redundancies that increase the cost of doing business. Well-thought-out, documented processes contribute significantly to a smooth operation. For more information on the importance of a policies and procedures manual, please see Chapter I. **Ω**

### Front-End Processing

#### *Accuracy of information*

Errors and omissions in data fields on the claim form—such as incorrect patient dates of birth, insurance addresses, insurance identification numbers, and patient addresses—frequently generate denials or requests for additional information. Both create duplicative work and delay reimbursement.

To reduce inaccurate claims, patient insurance information should be verified at the time of scheduling and re-verified at check-in. The receptionist or scheduler should always obtain a copy of the front and back of the patient's insurance card, which lets your office verify eligibility and benefits before the visit and recheck it before the patient is seen and the claim submitted. It's also useful to print out the patient's demographic and insurance information at check-in, have the patient confirm the information with his or her initials, and file that document with the chart.

### **Ω TOOLS**



A sample job description is available in the Appendix.

Some practice management systems will flag diagnosis codes not coded to the maximum specificity and modifier issues. If your system does not include this functionality, have staff spot-check claims for accuracy before they go out the door. While this is an additional step, the goal is to touch a claim once. When inaccurate information is submitted, it costs the practice money: Typically you receive either a denial or request for more information, which requires at least three times the staff time as it will to spot-check the claim in the first place. Beyond staff time, there's the cost of materials used to resend or appeal a claim, and of course, delays in payment itself are costly. According to the Advisory Board Company, a Washington-based research organization, an estimated 90 percent of claim denials are preventable and 67 percent of denials are recoverable. All additional staff time required to address these issues takes away from patient care.

### *Appointment Scheduling and Check-In*

Everyone feels the impact of an improperly managed appointment schedule—physicians, staff, and patients. If employees scheduling appointments are not properly trained in how to triage patients and allot the appropriate amount of time, either your schedule will have gaps or, more often, the physician will run behind, which frustrates patients. Before tackling a scheduling problem, take the time to look back at past schedules to recognize what isn't working and consider what might be a better approach. Then make a commitment to institute changes.

Begin by looking at a few weeks of your schedule to assess trends. Look for:

- Production
  - Number of patients seen each day
  - Average number of patients seen each day
  - New patients
  - Returning/established patients
- Number of double-booked appointments
- Number of patients “squeezed in”
- Number of no-shows
- Number of rescheduled appointments

Next, spend a few days gathering data on the amount of time spent with each patient. This is best done with a time flow study.

At the end of each day, staff should calculate the wait time between each step in the process. After completion of the time study assessment, determine the average wait time for each step. You may also want to consider asking your patients to complete similar time studies **Ω**

### **Ω TOOLS**



A sample time study assessment form is available in the Appendix.

The next step is to analyze the data and organize it into a report that identifies areas where your practice is doing well and areas where your practice needs improvement. For example, you may find that your physician is often running behind because the scheduler is routinely double booking, squeezing patients in, or not scheduling the appropriate amount of time for visits. In these cases, your practice should consider written protocols for your scheduler that clearly define conditions that require

immediate attention and should be fit into the schedule versus those that can be seen within a few days. Exceptions should only be made by the physician.

Another possibility is that your practice constantly runs behind because the physician is spending more time with patients than is actually scheduled. Practices can avoid this type of problem by creating written protocols that clarify how to triage a patient, including what types of questions to ask, as well as the appropriate amount of time to be scheduled depending on the patient's condition or reason for being seen. For example, pre- and post-op appointments may only require 10 or 15 minutes with the physician, whereas annual physical exams may require 60 minutes and new patient visits may require 30 minutes. If the scheduler is not asking the right questions to determine the appropriate amount of time to allot, the physician will be unable to stay on schedule.

Creating the right schedule for each practice requires an understanding of why it hasn't been achieved previously, predicting scheduling needs, and understanding how the physician works and what actions will promote a more efficient workday.

#### *Failure to collect*

Collecting co-pays, coinsurance, deductibles, and past due balances from the patient at the time of service is often one of the most overlooked tasks in a physician practice. Because the majority of payments come from managed care plans, it's easy to forget that charges paid directly by patients are an important source of revenue, and not collecting these up front can have significant impact. In fact, co-pays can represent as much as 20 percent of a physician's income.

Beyond impacts on the immediate revenue stream, billing after the fact runs up expenses as well. Studies have shown it can cost a practice at least \$5 to \$7 to generate a statement. So if a practice fails to collect a co-pay and later bills for it, the \$20 due turns into only \$13 to \$15. Multiply that by the number of patients you see in a day and it quickly adds up. And according to a U.S. Department of Commerce study, ability to collect drops to 74 percent at 90 days and to 30 percent at 180 days for payments not collected at the time of service.

Accordingly, consider implementing an office policy requiring that co-pays, coinsurance, deductibles, and unpaid balances be paid at the time of service. Clearly communicate this rule to patients before their visit and in your financial policy documents, which patients should sign annually. Inevitably, a handful of patients will show up without the cash, checkbook, or credit card to make the co-pay. Be prepared by locating the nearest ATM, and either direct the patient there or offer to reschedule the visit for a day when the patient can arrive with the proper payment. If you remain firm on this policy, it is unlikely that patients will forget the next time around. **Δ**

#### *Check-Out*

After examination, physicians frequently request that patients schedule follow-up, pre-op, or post-op visits for later dates. Failure to schedule these appointments at check-out can impact patient care and create additional work down the road in the form of scheduling-related phone calls.

Depending on the patient's type of insurance, coverage, and services performed, additional money may be due at check-out.

## **BALANCE BILLING**

California physicians should be aware that on January 8, 2009, the California Supreme Court put an end to the controversy surrounding "balance billing" of HMO enrollees in the emergency care context - the practice by out-of-network providers to bill patients the balance of an emergency care bill that the patients' Knox-Keene plan refused to pay. *The Court in Prospect Medical Group v. Northridge Emergency Medical Group*, \_\_ Cal.Rptr.3d \_\_, 2009 WL 36855 (2009) (Prospect), ruled that the Knox-Keene Act prohibits this practice of balance billing. The court clarified that providers may only seek recourse against the payors, not patients, for underpayments. The Department of Managed Health Care too has taken action to prohibit noncontracted providers from balance billing for emergency care services, promulgating a regulation, 28 C.C.R. sec. 1300.71.39, that defines such practices to be an "unfair billing pattern." The *Prospect* decision and the DMHC's regulation make it clear that balance billing for emergency care services is no longer permitted if the patient is covered by a Knox Keene-regulated plan (HMOs, certain PPOs, and any delegated medical groups or risk bearing organizations). For more information about the *Prospect* decision and its implications, see CMA's Balance Billing Toolkit at [www.cmanet.org](http://www.cmanet.org).

### **Δ NOTE**

Practices should exercise caution when it comes to turning patients away for failure to pay balances due at the time of service. Patients whose condition is urgent should be seen regardless.



To avoid these time-consuming and costly pitfalls, it is best practice to have a staff member available upon patient check-out to confirm with the patient whether any additional appointments need to be scheduled and to collect any additional payment due before the patient leaves the office.

#### *Late Arrivals and No-Shows*

Late arrivals create a backlog for the receptionist and delay the physician, which causes longer waits for other patients. No -shows can leave gaps in the schedule, reduce practice revenue, and contribute to timely access issues for patients.

To avoid these disruptions in your schedule and revenue flow and provide better access for your patients, it is a good idea to set some staff time aside each day to call patients to remind them of their appointments. Alternatively, an automated appointment confirmation and reminder service can help reduce missed appointments or late arrivals and relieve staff of the task of manually calling patients. Of course, automated services cost money, so run a cost-benefit analysis to see which option is right for your practice.

During the manual or automated confirmation call, patients should be reminded to bring their insurance card, patient history, and any other needed information for the visit. If your practice requires that a co-pay or deductible be paid at time of service, include that reminder as well. Clearly stating your practice's policies on late arrivals or no-shows is a good idea, too. For example, your office may require patients more than ten minutes late to reschedule, or specify that the physician may discharge a patient from the practice if he or she is a frequent no-show .

Before discharging a patient, be aware of state and federal laws regarding patient abandonment. ⚙️

#### *Managing call volume*

For many medical offices, the volume of phone calls received daily is overwhelming. Often the front desk is swamped with incoming calls, causing frustration and patient dissatisfaction. When call volume is heavy, inadequate phone systems may result in dropped calls or patients may hang up due to long waits. During peak periods patient calls may not be returned promptly and patients arriving for appointments may have to wait to check in because front desk staff are stuck on the phone. Some practices have responded by investing in expensive phone tree solutions, but frequently these only annoy patients rather than solve the volume issue.

Phones that ring off of the hook cause disruptions and contribute to staff errors. However, phone lines are still the most common form of communication in a practice, so understanding what makes your phone ring is extremely important to your practice's success.

Our physician practice volunteers decided to examine what made their phones ring. For two weeks they collected information about the nature of incoming calls (see sample tracking sheet in Appendix). Calls to schedule appointments comprised less than half the load; other calls were appointment cancellations, rescheduling, inquiries about lab results, requests for prescription refills, calls from pharmacies with formulary conflicts, requests for directions, questions about office policies, and insurance questions. Some were from repeat callers who were simply frustrated that they had not yet heard back from the physician or from the staff regarding the status of their prescription, authorization, or referral.

#### ON CALL

For more guidance on terminating the physician-patient relationship, see CMA ON CALL Document #0805, "Termination of the Physician-Patient Relationship." More information about patient discharge is typically available through your malpractice carrier.

One practice determined it was spending more than \$20,000 per physician per year trying to manage calls that could have been avoided. Instituting better processes and minimizing the need for patients to call improved patient satisfaction and reduced costs.

To reduce unnecessary call volume, you will need to systematically evaluate the reason for the calls and create a plan for change. Below is a list of call types and recommendations that can help you manage the call volume in your practice:

| Call Type                                | Recommendation                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
|------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Canceling or rescheduling an appointment | <ul style="list-style-type: none"> <li>• Implement an appointment reminder system (manual or automated) to contact patients 1 to 2 days prior to scheduled visit. Necessary rescheduling can be handled at the same time.</li> <li>• Have patients fill out an appointment reminder card at check-out and mail it to them 1 week in advance.</li> <li>• Develop a firm policy on no-shows and clearly communicate it to your patients.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
| Directions to the office                 | <ul style="list-style-type: none"> <li>• If making manual appointment reminder calls, confirm the patient knows where you are located. If not, provide general directions.</li> <li>• If using an automated appointment reminder service, the message to patients should include the general location of your office, including cross streets.</li> <li>• Post this information on your website (if applicable).</li> <li>• Include general directions in your new patient orientation package and send to patients before their scheduled visits.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
| Office policies                          | <ul style="list-style-type: none"> <li>• Mail new patients a full orientation package ahead of their scheduled visits. Include information on your office policies.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
| Clinical questions                       | <ul style="list-style-type: none"> <li>• Establish clear boundaries around clinical questions, such as when it is appropriate for office staff vs. a physician to answer a patient query. <ul style="list-style-type: none"> <li>- Calls requiring physician attention should be routed directly to physician along with the patient's chart, which is placed in a pre-designated spot.</li> </ul> </li> <li>• Establish callback hours, publish them in new-patient information materials, and remind patients of these when they call.</li> <li>• Follow through on a commitment made to contact a patient, even if you don't have the needed information or test results yet.</li> <li>• Consider implementing a secure system via your practice's website that allows patients to send messages to their physician and to request appointments, referrals, and prescriptions. This is a flexibility way for patients to relay a medical question at 10 p.m. and for physicians to respond when suitable or convenient.</li> </ul> |



| Call Type                              | Recommendation                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
|----------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Inquiries about lab/test results       | <ul style="list-style-type: none"> <li>• Develop an internal process for handling lab/test results and a policy on how results are communicated to the patient:</li> <li>• <b>Internal process:</b> <ul style="list-style-type: none"> <li>- Results that come in via mail or fax are placed in folder.</li> <li>- Charts are pulled, results are attached, and chart is placed in a designated spot for physician or practitioner review.</li> <li>- Reviewer signs off on the result and indicates the appropriate action (physician call, MA call, practitioner call, e-mail to patient, schedule an appointment, etc.) .</li> </ul> </li> <li>• <b>Make sure patients are aware:</b> <ul style="list-style-type: none"> <li>- How long it will generally take to receive results</li> <li>- How they will be notified</li> <li>- Who will notify them</li> <li>- When they should check back with the practice (if they haven't heard after x number of days, have additional complications, have questions, etc.)</li> <li>- Note: It is important to follow through on a commitment made to contact a patient, even if you don't have the needed information or test results yet.</li> </ul> </li> </ul> |
| Requests for prescription refills      | <ul style="list-style-type: none"> <li>• <b>Calls from patients:</b><br/>If your practice uses an automated phone system, consider including a message that encourages patients to contact their pharmacy for prescription refill requests.</li> <li>• <b>Calls from pharmacies:</b> <ul style="list-style-type: none"> <li>- Set aside a certain time of day to review routine refill requests received by fax or phone (e.g., during lunch hour and/or after 5 p.m.). Urgent requests should be handled based on patient needs.</li> <li>- Chart should be located, message or fax attached, and the chart placed in designated spot for physician review.</li> <li>- Pharmacy is contacted via phone/fax.</li> <li>- Patient should be contacted via phone/e-mail to advise them of the status of their prescription.</li> </ul> </li> <li>• Provide information about the prescription refill process via your website (if applicable).</li> <li>• Include this information in the orientation packages mailed to new patients.</li> <li>• Post this information in your waiting room.</li> </ul>                                                                                                          |
| Authorizations and referrals           | <ul style="list-style-type: none"> <li>• Explain the process for obtaining an authorization or referral to the patient: <ul style="list-style-type: none"> <li>- How long will it take</li> <li>- How and when will the patient be notified</li> <li>- When the patient should check back and with which practice (if they haven't heard after x number of days, etc.).</li> <li>- Note: It is important to follow through on a commitment made to contact a patient, even if you haven't yet obtained the authorization/referral.</li> </ul> </li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |
| Formulary conflicts/pharmacy questions | <ul style="list-style-type: none"> <li>• Physicians and their office staff have access to formulary reference software, such as Epocrates Rx (<a href="http://www.epocrates.com">www.epocrates.com</a>), free of charge. These types of products provide physicians with access to the formularies of most major payors. Verifying whether the medication your practice is about to prescribe is in the patient's formulary or requires an authorization can drastically decrease pharmacy callbacks.</li> <li>• Consider a dedicated fax line to receive all pharmacy inquiries/requests.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| Repeat callers                         | <ul style="list-style-type: none"> <li>• Clearly communicating when calls will be returned can decrease the number of repeat callers—for example, "calls will be returned within one business day." Handle urgent requests as needed.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |

Now it is time to identify what makes your practice's phone ring. We recommend using the call volume tracking sheet for one to two weeks. Any staff member responsible for answering the phone should use this form to track calls by frequency, time of day, and issue.. Note any additional issues in the space provided on your worksheet. Use one sheet per staff person per day. Total up the number of calls per day for a one-week period. At the end of the week, chart out the call volume for each topic. Identify the top five issues and determine if issues differ based on day of the week. **Ω**

You may also want to consider soliciting feedback from your patients regarding their satisfaction with your phone system. While in the waiting room, you can ask patients to complete a brief survey about their experience when contacting your office. **Ω**

Once you've reviewed and addressed the volume of calls, assess whether your practice has sufficient phone line capacity to handle it. Contact your phone company to request a report on busy signals, abandoned calls, and peak call volume. Typically there is a fee for this type of information, but it is an inexpensive way to help your practice assess whether you need to add lines or make changes that will improve customer satisfaction and access.

## **Avoiding Common Inefficiencies**

### *The Physician's Time*

A practice's biggest expense is the physician. The goal is to be efficient and productive and to make sure the physician is using his time and other resources wisely.

### *Schedule*

Your major source of revenue is your patients. Regardless of your financial mode of practice (capitation, contracted fee for service, non-contracted self-pay), your goal should be to have a full schedule of patients each day and handle that schedule effectively.

### *Delegate*

Many physicians complain that they have too many meetings to attend, too much business correspondence to read and review, and too much paperwork to complete. Delegate these administrative tasks. Let your office manager start doing some real management. If that employee doesn't have the necessary management education, investing in such education will ultimately save you time and money.

### *Organize and Prioritize*

Use a task organizer, such as a Day Runner, Microsoft Outlook task list, or a simple handwritten to-do list, to plan, organize, and prioritize your non-patient appointment tasks. By prioritizing your non-patient tasks, you can expect to save 30 minutes a day and use the rest of your time more productively. For example, if you have prioritized your tasks and there is a no-show, you can use that time to accomplish one of the tasks on your list rather than wasting time.

Perform a self-inventory to determine how well you manage this list. Over a period of a week or 10 days, look at how many of those tasks in your organizer are actually being accomplished, how many are being dismissed or forgotten, and how many are being bumped into another day. If you are accomplishing less than 90 percent of

## **Ω TOOLS**



A sample call volume tracking sheet is available in the Appendix.

## **Ω TOOLS**



A sample patient satisfaction survey is available in the Appendix.

# 6

## Practical Steps Practices Can Take to Ensure HIPAA Compliance

By David Ginsberg, PrivaPlan Associates

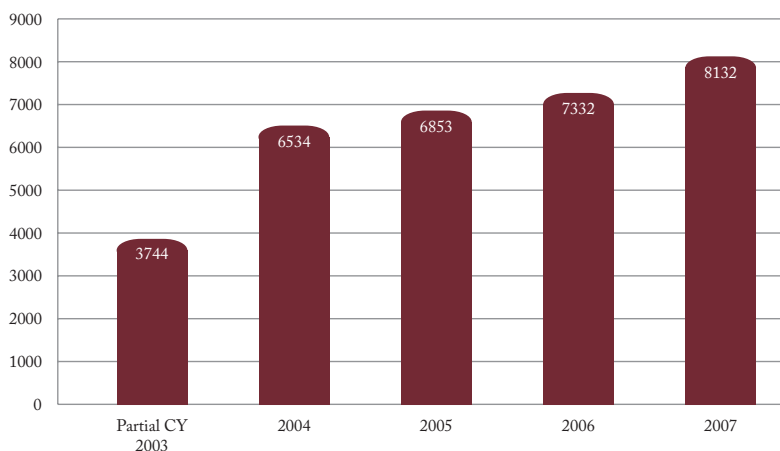
Most medical practices feel they have done all they need to do to satisfy Health Insurance Portability and Accountability Act (HIPAA) requirements and are reluctant to dedicate precious resources to additional compliance efforts. However, a number of gaps can expose medical practices to patient identity theft and violation of state laws that may be far stricter than HIPAA requirements.

Rather than reviewing HIPAA regulations in depth, this chapter provides an overview of information that will help you remain HIPAA compliant.

### HIPAA ENFORCEMENT

HIPAA enforcement is real. The latest data from the Office of Civil Rights (OCR) show that complaints and investigations are increasing. As seen in Figure 1 below, HIPAA complaints have increased since 2003 by 117%. Most complaints are initiated by disgruntled employees.

Figure 1 - Health Information Privacy Complaints Received by Calendar Year



### HIPAA COMPLIANCE RESOURCE

For an in-depth guide to HIPAA compliance, medical practices can purchase a complete do-it-yourself tool for HIPAA compliance developed by the California Medical Association (CMA) and its HIPAA partner, PrivaPlan Associates, at [www.privaplan.com](http://www.privaplan.com). Physicians in other areas should contact their state and local medical associations or PrivaPlan for more specific information about HIPAA compliance.

For these reasons, it is more critical than ever for physicians to review their current policies and procedures and upgrade them, if necessary.

Federal law does not create what is known as a private cause of action under HIPAA. In other words, individuals cannot sue for a privacy or security violation citing the HIPAA regulation. Only the federal government can enforce HIPAA and take covered entities to court for violations. However, some states have allowed private parties to bring actions seeking remedies for violations of HIPAA.

So, while HIPAA enforcement has been driven by complaints to either OCR or the Centers for Medicare & Medicaid Services (CMS), these cases have opened the door for successful private lawsuits against physicians when a privacy or security violation occurs.

## HOW CAN YOU LOWER YOUR RISK?

The best defense against a HIPAA-related action is to not have a privacy or security violation occur. Here are the minimum steps any HIPAA-covered entity should take:

1. Periodically review your HIPAA privacy and security compliance efforts.
2. Ensure your policies and procedures are up-to-date.
3. Ensure that your policies and procedures actually “work,” are understood by employees, and are implemented.
4. Ensure your training is up-to-date for all employees, board members, key contractors, etc. Employees should be required to annually review and sign a statement that they have read and understand the office’s HIPAA privacy policies. (This information should be stored in personnel files.)
5. Ensure that key procedures are in place (such as the complaint procedure).
6. Ensure that your business associates have written agreements in place.
7. Ensure that you report and respond to any and all privacy and security incidents.
8. Ensure that your workforce and patients understand they will not be retaliated against if they complain about or notify you of a privacy or security breach.

## MEDICAL IDENTITY THEFT

Medical identity theft is on the rise. In some cases protected health information is stolen to submit fraudulent claims; in others the information is being used to obtain health care coverage itself (i.e., the identity of an insured individual is assumed). And the risk comes not only from outside sources such as hackers. You must also ensure that sensitive patient data is available only to staff who need to access that data.

### Some Practical Steps You Can Take

#### *Establish (and Follow) Workforce Clearance Procedures*

It has become increasingly important to do effective criminal background checks on employees who will have access to protected health information. Be sure to follow state and federal laws regarding how you notify a new employee of an impending background check and how you apply the findings.

#### *Develop Effective Workforce Access and Authorization Protocols*

In the “old days” it would take a large truck to steal information on even a small solo practice’s patients. Today it requires a USB thumb drive and a few minutes. As more and more organizations convert to electronic health records and use portable devices, this threat becomes greater.

Are employees restricted to accessing only the information needed for their jobs? If not, most practice management systems have security features that will allow you to limit access by user. We strongly recommend contacting your vendor to find out how to use this feature.

#### *Establish Effective Workforce Termination Procedures*

Policies should be in place to terminate all access to protected health information, including systems and building access, immediately upon the termination of an

employee. Policies should be in place to discourage the sharing of system passwords. If you provide staff with keys to your office, make sure each key is clearly stamped “Do not duplicate.” This will alert locksmiths not to make duplicate keys. Finally, be cautious when giving employees or others access to your Medicare and Medicaid provider transaction access numbers (PTAN). There have been many cases where these numbers have been acquired fraudulently to submit bogus claims. This also applies to other personal information that can be used to obtain Medicare and Medicaid provider numbers. If you suspect your provider number has been stolen, report it immediately. Check with your local Medicare and Medicaid fiscal intermediary about how to report fraud.

#### *Routinely Review System Activity*

It is important to routinely review system activity and conduct technical audits to monitor suspicious activity. Your practice management system should have auditing capabilities to track employee activity in patient accounts. You may not have doubts about the integrity of your staff, but even trusted staff may be inappropriately accessing/using patient information. Schedule enough time every month to go over reports with your office manager or administrator. Make sure you understand the data and ask questions if you don’t.

#### *Maintain Data and Equipment in an Encrypted Mode*

All electronic devices and data should be password protected to prevent theft.

#### *Use Security Reminders*

Use periodic security reminders and alerts to keep your workforce vigilant and on the lookout for security incidents.

These steps are, of course, just part of your overall HIPAA compliance program. Make certain your organization has done everything it can to protect sensitive data.

## **COMPLIANCE REVIEWS AND INVESTIGATIONS**

In the case of a complaint or investigation, HIPAA requires cooperation from covered entities, sometimes including allowing investigators access to facilities, records, and other information at any time, without notice. **Ω**

## **NOTICE OF PRIVACY PRACTICES**

All physicians covered by HIPAA are required to provide their patients with a written notice of the privacy practices (NPP) they use to protect patients’ health information. Covered physicians that maintain a physical delivery site must post their privacy practices in a prominent place likely to be seen by patients.

HIPAA also requires that providers with a “direct treatment relationship” use their best effort to have the patient sign an acknowledgement of receipt.

If you at some point revise your privacy practices, you need only make the revised version available *upon request* (and of course replace your existing posted NPP as well as the one you provide to new patients). You do not need to resend the revised NPP to all existing patients.

### **Ω TOOLS**



A list of information that might be requested in a HIPAA investigation or compliance review is included in the Appendix.

Also remember that if you have a website, you must prominently post the NPP on your website and make it available to viewers who request a copy.

Sample NPPs in English and Spanish are available as part of PrivaPlan's HIPAA compliance toolkit.

## PROTECTING INDIVIDUALS WHO COMPLAIN

Covered entities should be especially vigilant when handling complaints so there is not the impression of retaliation. Such a signal may not always be obvious to you, but to your employees or patients who complain, sometimes even subtle and unrelated actions can feel like retaliation. For example:

- Rescheduling patients who have complained or “passing them off” to other providers
- Disciplining an employee who has complained for an unrelated workplace action

### Some Practical Tips

1. Be sure you have written policies and procedures and that every member of your workforce has been trained in these procedures.
2. Review your current Notice of Privacy Practices and be sure it clearly states that the individual will not be penalized or retaliated against for filing a complaint.
3. Review your complaint and whistleblower policies and procedures. The CMA/PrivaPlan HIPAA Privacy and Security Compliance Toolkit contains appropriate language for this. The toolkit is available for purchase at [www.privaplan.com](http://www.privaplan.com).
4. Whenever a patient or member of the workforce files a complaint, immediately ensure that your key managers, owners, and other relevant supervisors understand they should be careful not to act in a way that can be interpreted as retaliatory or intimidating.
5. Of course, handle complaints immediately and with full documentation.
6. If you find you have legitimately violated HIPAA, implement a corrective action plan. ■



**SAMPLE FORM: Call Volume Tracking Sheet**

Day of Week: \_\_\_\_\_

**Time of Day**

| Issue                         | 9-10am | 10-11am | 11am-12pm | 12-1pm | 1-2pm | 2-3pm | 3-4pm | 4-5pm | Total |
|-------------------------------|--------|---------|-----------|--------|-------|-------|-------|-------|-------|
| Scheduling                    |        |         |           |        |       |       |       |       |       |
| Rescheduling                  |        |         |           |        |       |       |       |       |       |
| Authorizations                |        |         |           |        |       |       |       |       |       |
| Referrals                     |        |         |           |        |       |       |       |       |       |
| Labs/test results             |        |         |           |        |       |       |       |       |       |
| Rx refills                    |        |         |           |        |       |       |       |       |       |
| Billing questions             |        |         |           |        |       |       |       |       |       |
| Questions for nurse/physician |        |         |           |        |       |       |       |       |       |
| Patient demographics          |        |         |           |        |       |       |       |       |       |
| Forms                         |        |         |           |        |       |       |       |       |       |
| Labs                          |        |         |           |        |       |       |       |       |       |
| Pharmacy                      |        |         |           |        |       |       |       |       |       |
| Health Plan                   |        |         |           |        |       |       |       |       |       |
| Directions                    |        |         |           |        |       |       |       |       |       |
| Repeat calls                  |        |         |           |        |       |       |       |       |       |
| Other                         |        |         |           |        |       |       |       |       |       |
| Total                         |        |         |           |        |       |       |       |       |       |